

UAE Paramedic Case Generator

Session Summary Report

Generated: 16/09/2026, 07:06:51

Case Information

Case Title: Severe Allergic Reaction Restaurant
Category: respiratory
Priority: CRITICAL
Complexity: BASIC
Year Level: 1st-year
Patient: 32yo female
Location: Seafood restaurant, Al Maryah Island, Abu Dhabi

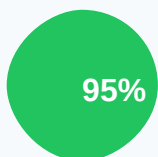
Dispatch Information

Call Reason: 32-year-old female difficulty breathing after eating at restaurant, swollen face
Time of Day: evening
Caller Info: Restaurant manager (anxious)

Initial Vital Signs

BP: 85/50 | Pulse: 125 bpm | RR: 28/min | SpO2: 93% | GCS: 15/15 | Glucose: 6.2 mmol/L

Performance Summary



Excellent

125 / 125 points

Time: 08:42

Base score: 100%

-5% Inappropriate: Naloxone 0.4mg — Naloxone used without clear opioid physiology.

Adjusted score: 95%

Performance Breakdown

Overall: Excellent (95%)

Excellent performance. Systematic, thorough assessment with the decisive intervention given early.

Assessment & Recognition	96%
Systematic ABCDE; key findings identified.	
Clinical Management	94%
Adrenaline-first, oxygen and fluids supported.	
Timing & Efficiency	90%
Prompt first intervention.	
Patient Safety	100%
No contraindicated or allergenic drugs given.	

Strengths

- + Strong systematic assessment (ABCDE).
- + Interventions improved oxygenation.

Focus Next

- > Reassess after each intervention.

Transport & Handover

Priority	Lights & Sirens
Position	Sitting Upright
Pre-alert	Yes Nearest ED
Working Dx	Anaphylaxis
Arrival on scene	00:18:05
Departed (handover)	00:26:47
On-scene time	08:42
Time to first Rx	1:12

Completed Actions

+ Scene Safety & BSI (Critical)
+ Assess airway patency (Critical)
+ Assess breathing rate (Critical)

Key Learning Points

- 1 Anaphylaxis = acute multi-system allergic reaction involving airway/breathing AND/OR circulation skin signs alone are NOT anaphylaxis
- 2 IM adrenaline (0.5mg / 500mcg for adults) is the ONLY first-line life-saving treatment give it IMMEDIATELY
- 3 Inject into the anterolateral thigh (vastus lateralis) NOT the deltoid, NOT subcutaneously
- 4 Adrenaline works within minutes: vasoconstriction (raises BP), bronchodilation (relieves wheeze), reduces oedema
- 5 Can repeat IM adrenaline every 5 minutes if no improvement there is no maximum dose in anaphylaxis
- 6 Antihistamines (chlorphenamine) treat itch and urticaria only they do NOT treat airway or cardiovascular collapse
- 7 Positioning matters: flat + legs up for shock; sitting up if respiratory distress predominates
- 8 ALL anaphylaxis patients MUST go to hospital risk of biphasic reaction (recurrence hours later)
- 9 Common triggers in UAE: shellfish, nuts, bee/wasp stings, medications (antibiotics, NSAIDs)
- 10 Always ask: has the patient used their EpiPen? Do they have one? (This patient left it at home)
- 11 Anaphylaxis vs allergic reaction: if any airway compromise, breathing difficulty, or hypotension -> it is anaphylaxis

Applied Treatments & Interventions

1. IM adrenaline matched to anaphylaxis

Applied at: 07:06

2. Oxygen matched to hypoxia

Applied at: 07:06

3. Bronchodilator matched to wheeze

Applied at: 07:06

4. Naloxone used without clear opioid physiology

Applied at: 07:06

Management Summary

Immediate

- Scene safety and BSI
- Remove allergen exposure (move patient away from food)
- Call for help early
- IM Adrenaline 0.5mg (500mcg) into anterolateral thigh THIS IS THE PRIORITY
- High-flow oxygen 15L/min via non-rebreather mask
- Lay patient flat with legs elevated (if tolerated may need to sit up if severe wheeze)
- Monitor airway continuously for deterioration

Definitive

- Reassess after 5 minutes repeat IM adrenaline if no improvement
- IV access and IV fluid bolus (500ml-1L normal saline)
- Transport to hospital all anaphylaxis patients need observation (biphasic risk)
- Antihistamine (chlorphenamine 10mg IM) adjunct only, NOT a substitute for adrenaline
- Hydrocortisone 200mg IV prevents biphasic reaction (takes hours to work)
- Minimum 6-hour observation in hospital post-anaphylaxis

Monitoring

- Continuous airway assessment listen for stridor development
- SpO2 continuous monitoring
- Blood pressure every 2-3 minutes initially
- Heart rate and rhythm
- Respiratory rate and work of breathing
- Level of consciousness
- Rash progression or resolution
- Watch for biphasic reaction (can occur up to 72 hours later)

Transport

- Pre-alert hospital: anaphylaxis, adrenaline given, vital signs
- Transport with ongoing monitoring
- Have second dose of adrenaline ready
- Position: flat with legs elevated OR sitting up if respiratory distress predominates
- Rapid transport do not delay on scene

Vital Signs Summary

	BP	HR	SpO2	RR
Initial	85/50	125	93%	28
Final	110/70	100	97%	18

Snapshots recorded: 2

Common Pitfalls to Avoid

- (!) DELAYING adrenaline this is the #1 error. Give adrenaline FIRST, not antihistamines
- (!) Giving antihistamines instead of adrenaline antihistamines do NOT treat anaphylaxis
- (!) Wrong route: giving adrenaline IV instead of IM (IV adrenaline can cause fatal arrhythmias in non-arrest patients)
- (!) Wrong site: injecting into deltoid or buttock instead of anterolateral thigh
- (!) Not repeating adrenaline if first dose does not work within 5 minutes
- (!) Standing the patient up this can cause cardiac arrest in anaphylaxis (empty ventricle syndrome)
- (!) Not monitoring the airway angioedema can progress to complete airway obstruction
- (!) Forgetting to remove the allergen (move patient away from the prawns)
- (!) Not transporting to hospital biphasic reactions can be fatal
- (!) Confusing anaphylaxis with panic attack always check for skin signs, wheeze, and hypotension

Further Study Resources

Guidelines

- [World Allergy Organization] [World Allergy Organization Anaphylaxis Guidance](#) *
- [Resuscitation Council UK] [Resuscitation Council UK: Anaphylaxis Algorithm](#) *

Articles

- [REBEL EM] [Anaphylaxis - Evidence Review](#) *

Resources from: NICE, Resuscitation Council UK, Radiopaedia, EMDocs, REBEL EM, ALiEM, EM Cases, EMCrit.